Two HRS Core cognitive classification methods informed by HRS/HCAP

Richard N. Jones

Jennifer J. Manly

Douglas Tommet

Lindsay Kobayashi

Lindsay H. Ryan

Ryan J. McCammon

Steven G. Heeringa

David Weir

Kenneth M. Langa

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Abstract

- **Importance:** population cognitive classification in HRS Core; dementia and MCI surveillance; limited assessment battery
- **Objective:** describe two HRS Core methods calibrated to HRS/HCAP (Core algorithm, PMM); compare with the HRS/HCAP Algorithm, consensus, Langa-Weir, and Hudomiet
- **Design:** 2016 HRS Core classification in HRS/HCAP participants; consensus-panel secondary comparator
    - Study sample: HRS/HCAP 2016 participants, N = 3,496
    - [Provenance: R/MS_MAIN/MS_001_analysis_cohort.R]
- **Methods:** Core algorithm; PMM
    - Primary comparator: HRS/HCAP Algorithm
    - Secondary comparators: (1) HCAP consensus; (2) Langa-Weir; (3) Hudomiet classification
    - Statistical analysis: dementia versus non-dementia kappa; 3-class quadratic weighted kappa (Normal, MCI, Dementia); all statistics other than frequencies use complex sampling weights
    - [Provenance: R/MS_MAIN/MS_010_results_objects.R]
- **Results:**
    - Dementia/non-dementia kappa
        - Core algorithm versus HRS/HCAP Algorithm: 0.353
        - PMM versus HRS/HCAP Algorithm: 0.429
    - 3-class kappa
        - Core algorithm versus HRS/HCAP Algorithm: 0.505
        - PMM versus HRS/HCAP Algorithm: 0.509
    - [Provenance: R_objects/A7_100_hcap_tables.rds; R/PMM_110_Comparison_of_Consensus_Langa_Weir.R]
- **Conclusions and relevance:** Two core classification approaches; agreement evidence rather than clinical validation; uncertainty in distinguishing MCI from normal cognition and dementia; limited precision of comparisons with consensus diagnoses because the consensus-panel subsample is small
    - [Provenance: analysis design; inference]

Key terms

- **HRS/HCAP Algorithm**: c.f. Manly, Jones et al. (2022)
- **Core algorithm**: HCAP algorithm adapted to the HRS Core data

- **PMM**: Profile mixture model using measures collected in the HRS Core, calibrated to the HRS/HCAP Algorithm and scored in the HRS/HCAP sample

Introduction

- Population cognitive classification from brief HRS Core assessments
    - Prevalence estimation; etiologic research; trial sampling; policy surveillance
    - Core assessment constraints; incomplete cognitive testing; proxy/informant information
- HRS/HCAP Algorithm as primary comparator
    - HCAP neuropsychological assessment; actuarial Normal/MCI/Dementia classification
    - HRS/HCAP Algorithm; primary HCAP-based comparator
    - Manly et al. (2022); Farron et al. (2025)
    - [Provenance: R/A7_100-comparison_of_diagnoses.R; existing manuscript materials]
- Study aim
    - Core algorithm and PMM; HRS/HCAP Algorithm primary comparator
    - Secondary comparators: HCAP consensus; Langa-Weir; Hudomiet
    - Classification margins; 3-class agreement; dementia/non-dementia agreement

Methods

Design and analytic samples

- Study sample: HRS/HCAP 2016 participants, N = 3,496
    - All HCAP participants; no Core cognitive-test requirement
    - Participants without Core cognitive testing classified from informant data (Jorm IQCODE): Core algorithm Jorm fallback; PMM Jorm model
    - Agreement-statistic samples: Core algorithm N = 3,496; PMM N = 3,496
    - [Provenance: R/MS_MAIN/MS_001_analysis_cohort.R; R_objects/A7_005_hrs16_merged.rds; R/A7_100-comparison_of_diagnoses.R; R/PMM_110_Comparison_of_Consensus_Langa_Weir.R]
- Consensus panel subset
    - Validation-subsample design; inverse-probability weighting to account for selection into the consensus panel
    - [Provenance: R/A7_100-comparison_of_diagnoses.R; R/PMM_110_Comparison_of_Consensus_Langa_Weir.R]

HRS Core classification approaches

- Two Core classification approaches
    - Core algorithm
        - Survey-weighted CFA; demographically normed cognitive score; cognitive/self-report/informant rules

        - [Provenance: R/A7_050-mplus_model.R; R/A7_055-norm_factor_score.R; R/A7_075-implement_algorithm.R]
    - PMM
        - HRS/HCAP-calibrated probabilistic Normal/MCI/Dementia classification; combined cognition and Jorm models (i.e., two PMM models, one trained using cognition data, the other trained using informant data (Jorm IQCODE)).
        - *Limitation*: The HRS/HCAP IQCODE look back window is the past 10 years, the Core IQCODE has a look back window of 2 years.
        - [Provenance: R/PMM_103_calibration_model.R; R/PMM_110_Comparison_of_Consensus_Langa_Weir.R]

- Single-factor Core cognition model
    - Not enough items to separately score 5 domains matching Manly et al. (2022)
    - Initial 8-indicator, complex-survey model fit evaluated with WLSMV
    - Factor scores estimated with MLR, a probit link, and EAP scoring after fixing item parameters to the MLR model estimates
    - Factor-score normalization; age, sex, race/ethnicity, education covariates
    - [Provenance: R/A7_050-mplus_model.R; mplus_output/A7/model_1a_fixed/model_1a_fixed.inp; R/A7_055-norm_factor_score.R]
- Rule-based classification
    - Cognitive thresholds; self-reported concern; JORM for missing cognitive assessments
    - Normal, MCI, Dementia output
    - More detail in **Appendix 1**
    - [Provenance: R/A7_075-implement_algorithm.R]

- HRS/HCAP calibration
    - Known-class HRS/HCAP Algorithm classification; cognitive, functional, self-report, informant indicators
    - No latent variables for overall cognition, individual tests used
    - Calibration parameters fixed for scoring; scored in the same HRS/HCAP sample used for calibration (in-sample classification); not yet applied to the HRS Core sample outside HCAP
    - [Provenance: R/PMM_100_norming_npb.R; R/PMM_103_calibration_model.R]
- Class proportions (priors)

    - Class-specific parameters estimated with HCAP16WGTR; MLR estimation, Bayes-rule posterior classification (plug-in rule; McLachlan, 1992)
    - Class proportions fixed at the HCAP16WGTR-weighted HRS/HCAP Algorithm distribution, computed outside Mplus; Mplus KNOWNCLASS estimates class proportions from unweighted counts
        - Cognition model: Normal 69.4%, MCI 21.4%, Dementia 9.2%
        - Jorm model: Normal 31.7%, MCI 31.6%, Dementia 36.7%
    - [Provenance: R/PMM_101_mplus_function.R; R/PMM_103_calibration_model.R; R/PMM_104_weight_check.R; mplus_output/pmm_103/pmm_hcap_103b.out; mplus_output/pmm_103_jorm/pmm_hcap_103b_jorm.out]
- Combined classification
    - Cognition-model probabilities; JORM-model probabilities when available
    - Probabilistic Normal/MCI/Dementia membership; replicated probability-weighted comparisons
    - More detail in **Appendix 2**
    - [Provenance: R/PMM_110_Comparison_of_Consensus_Langa_Weir.R]

Comparison classifications and estimands

- Primary comparator: HRS/HCAP Algorithm
    - HCAP population weights: HCAP16WGTR
    - [Provenance: R/A7_100-comparison_of_diagnoses.R]
- Secondary comparators
    - HCAP consensus diagnosis: normalized HCAP16WGTR x samplingP^-1 weights
    - Langa-Weir; Hudomiet classification
    - [Provenance: R/A0_017-recode_classification.R; R/A7_100-comparison_of_diagnoses.R]
- Agreement estimands
    - 3-class quadratic weighted kappa
    - Dementia versus non-dementia kappa
    - Marginal Normal/MCI/Dementia distributions
    - [Provenance: R/MS_MAIN/MS_010_results_objects.R]

Results

Analytic samples and classification margins

- Table 1: HRS/HCAP study sample characteristics
    - Dynamic counts and descriptive statistics

    - [Provenance: R/MS_MAIN/MS_Tab_Fig_Apndx-030-Table-1.qmd]
- Table 2: weighted classification margins
    - HRS/HCAP Algorithm; Core algorithm; PMM; Langa-Weir; Hudomiet
    - HCAP16WGTR-weighted margins
        - HRS/HCAP Algorithm: Normal 69.4%, MCI 21.4%, Dementia 9.2%
        - Core algorithm: Normal 71.0%, MCI 20.7%, Dementia 8.3%
        - PMM: Normal 76.7%, MCI 13.6%, Dementia 9.7%
    - [Provenance: R/MS_MAIN/MS_010_results_objects.R]
- Figure 1: Core algorithm
    - Classification rules and participant counts
    - [Provenance: R/MS_MAIN/MS_Tab_Fig_Apndx-020-Figure-1.qmd]
- Figure 2: PMM class-probability distributions
    - Probability separation; uncertainty near class boundaries
    - [Provenance: R/PMM_112_Margins.R]

Agreement with HRS/HCAP Algorithm

- Core algorithm versus HRS/HCAP Algorithm: 3-class kappa 0.505; binary kappa 0.353
    - [Provenance: R_objects/A7_100_hcap_tables.rds]
- PMM versus HRS/HCAP Algorithm: 3-class kappa 0.509; binary kappa 0.429
    - [Provenance: R/PMM_110_Comparison_of_Consensus_Langa_Weir.R]
- Table 3: pairwise agreement, comparators, weight definitions
    - [Provenance: R/MS_MAIN/MS_010_results_objects.R]

Agreement with consensus and established HRS classifications

- Core algorithm, PMM, Langa-Weir, Hudomiet
    - Separate HCAP population and consensus-subsample estimands
    - Weight-dependent interpretation
    - [Provenance: R/MS_MAIN/MS_010_results_objects.R]
- Table 4: A7 pairwise three-class agreement matrix
    - [Provenance: R_objects/A7_100_hcap_tables.rds]

Discussion

- Two HRS Core classification approaches
    - Rule-based Core algorithm; probabilistic PMM classification
    - Same HRS/HCAP learning target; distinct measurement and classification assumptions

    - [Provenance:analysis methods]
- Primary comparison
    - HRS/HCAP Algorithm; agreement and marginal-distribution evidence
    - [Provenance: Tables 2-4]
- Secondary comparisons
    - Consensus: small validation subset; inverse-probability weights
    - Langa-Weir and Hudomiet: established HRS comparators; different development targets
    - [Provenance: R/A7_100-comparison_of_diagnoses.R]
- Interpretation limits
    - Classification agreement; no clinical diagnostic validation outside HCAP consensus subset
    - MCI boundary; model-dependent and threshold-dependent assignments
    - PMM agreement with the HRS/HCAP Algorithm is in-sample (apparent error rate; McLachlan, 1992); expected to be optimistic relative to application in new samples
    - [Provenance: analysis design; inference; R/PMM_103_calibration_model.R]

Two HRS Core cognitive classification methods informed by HRS/HCAP: Figures, tables, and appendices

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Table 1. Participant Characteristics

Characteristic	HRS/HCAP 2016 participants (N = 3,496)
Sex (N (%))	
Female	2,095 (59.9%)
Male	1,401 (40.1%)
Age in years (Mean (SD))	76.0 (7.6)
Race (N (%))	
White	2,755 (78.8%)
Black	563 (16.1%)
All other groups	177 (5.1%)
Not collected	1 (0.0%)
Hispanic origin (N (%))	
Not Hispanic	3,111 (89.0%)
Hispanic	383 (11.0%)
Not obtained	2 (0.1%)
Years of education (Mean (SD))	12.7 (3.2)
HRS/HCAP Classification (N (%))	
Normal	2,313 (66.2%)
Mild cognitive impairment	808 (23.1%)
Dementia	375 (10.7%)

Table 1. Participant Characteristics

Characteristic	HRS/HCAP 2016 participants (N = 3,496)
Langa-Weir Classification (N (%))	
Normal	2,432 (69.6%)
Cognitively impaired, not demented	731 (20.9%)
Dementia	333 (9.5%)
Jorm IQCODE score from Core (Mean (SD))	3.8 (0.8)
Jorm IQCODE score from Core (missing)	3,283

Source: R/MS_MAIN/MS_001_analysis_cohort.R; R_objects/A7_005_hrs16_merged.rds.

Jorm IQCODE was collected in the HRS Core interview, not HCAP, and only from participants who did not complete Core cognitive testing.

Notes: The study sample includes all 3,496 HRS/HCAP 2016 participants, including participants without Core cognitive testing who were classified from informant (Jorm IQCODE) data. Race and ethnicity data were gathered via self-report at the time of first interview and are considered to be markers of exposure to evolving systems of racism, not as a proxy for genetic variation or any other biological variable. Race was self-selected by participants at the time of the first interview from a list of options defined by the 2000 US Census criteria. Other race includes a pooled group of participants who identified as American Indian or Alaska Native, Asian, Native Hawaiian or Pacific Islander, or another self-described race, consolidated due to small sample sizes and risk of identification.

Table 2. HCAP classification margins by method

	Normal		MCI		Dementia	
Method	N	Weighted %	N	Weighted %	N	Weighted %
HRS/HCAP Algorithm	2,313	69.4%	808	21.4%	375	9.2%
Core algorithm	2,433	71.0%	747	20.7%	316	8.3%
PMM	2,557	76.7%	568	13.6%	371	9.7%
Secondary comparators						
Langa-Weir	2,432	74.9%	731	17.3%	333	7.9%
Hudomiet	2,326	71.1%	865	21.0%	286	7.2%

Note. N values are unweighted HRS/HCAP participant counts. Percentages use HCAP16WGTR; PMM counts and percentages use the non-replicated combined cognition/Jorm model sample.

References. Manly, J. J., Jones, R. N., Langa, K. M., Ryan, L. H., Levine, D. A., McCammon, R., Heeringa, S. G., & Weir, D. (2022). Estimating the prevalence of dementia and mild cognitive impairment in the US: The 2016 Health and Retirement Study Harmonized Cognitive Assessment Protocol Project. JAMA Neurology, 79(12), 1242-1249. https://doi.org/10.1001/jamaneurol.2022.3543

Langa, K. M., Weir, D. R., Kabeto, M., & Sonnega, A. (2023). Langa-Weir Classification of Cognitive Function (1995-2020). https://hrsdata.isr.umich.edu/sites/default/files/documentation/data-descriptions/1695907706/Data_Description_Langa_Weir_Classifications2020_V2.pdf

Hudomiet, P., Hurd, M. D., & Rohwedder, S. (2022). Trends in inequalities in the prevalence of dementia in the United States. Proceedings of the National Academy of Sciences, 119(46), e2212205119. https://doi.org/10.1073/pnas.2212205119

Table 3. Agreement among Core classifications and comparison diagnoses

Classification 1	Classification 2	Three-class quadratic weighted kappa	Dementia versus non-dementia kappa
Core Algorithm	HRS/HCAP Algorithm	0.505	0.353
PMM	HRS/HCAP Algorithm	0.509	0.429
Langa-Weir	HRS/HCAP Algorithm	0.510	0.407
Hudomiet	HRS/HCAP Algorithm	0.551	0.457
HCAP Consensus	HRS/HCAP Algorithm	0.728	0.536
Core Algorithm	Langa-Weir	0.560	0.435
PMM	Langa-Weir	0.575	0.480
Hudomiet	Langa-Weir	0.717	0.670
HCAP Consensus	Langa-Weir	0.099	0.736
Core Algorithm	Hudomiet	0.605	0.474
PMM	Hudomiet	0.623	0.529
HCAP Consensus	Hudomiet	0.396	0.806
Core Algorithm	HCAP Consensus	0.284	0.675
PMM	HCAP Consensus	0.486	0.464

Source: A7 rows: R_objects/A7_100_hcap_tables.rds, generated by R/A7_100-comparison_of_diagnoses.R. PMM rows: R/PMM_110_Comparison_of_Consensus_Langa_Weir.R; PMM-Hudomiet was calculated from the same saved probabilities and A7 comparison data. Three-class values are quadratic weighted kappas; binary values are kappas for dementia versus non-dementia. Consensus comparisons use normalized HCAP16WGTR / samplingP weights; other comparisons use HCAP16WGTR.

Table 4. Pairwise three-class agreement among HCAP comparison methods

method	Consensus diagnosis	Algorithmic diagnosis	Langa-Weir	HRS classification model	Hudomiet
Consensus diagnosis	1.000	0.728	0.099	0.284	0.396
Algorithmic diagnosis	0.728	1.000	0.510	0.505	0.551
Langa-Weir	0.099	0.510	1.000	0.560	0.717
HRS classification model	0.284	0.505	0.560	1.000	0.605
Hudomiet	0.396	0.551	0.717	0.605	1.000

Source: Source: R_objects/A7_100_hcap_tables.rds, generated by R/A7_100-comparison_of_diagnoses.R. Entries are quadratic weighted kappas. Consensus comparisons use normalized HCAP16WGTR / samplingP weights; other comparisons use HCAP16WGTR.

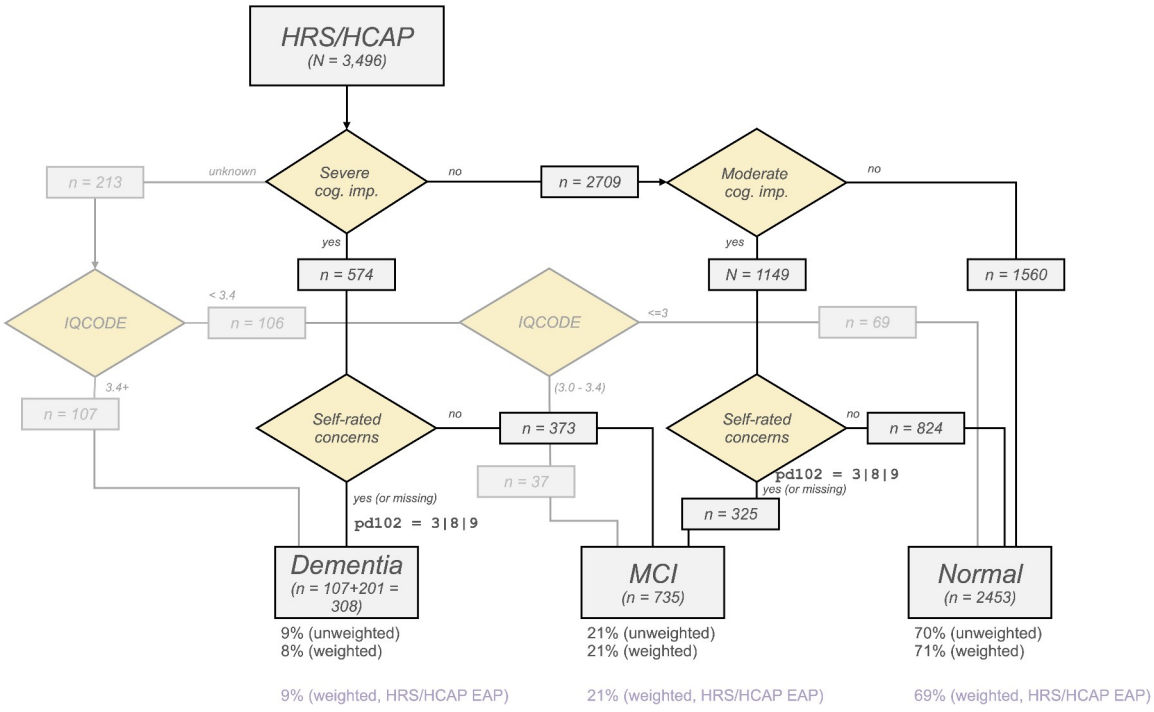


Figure 1: Core algorithm

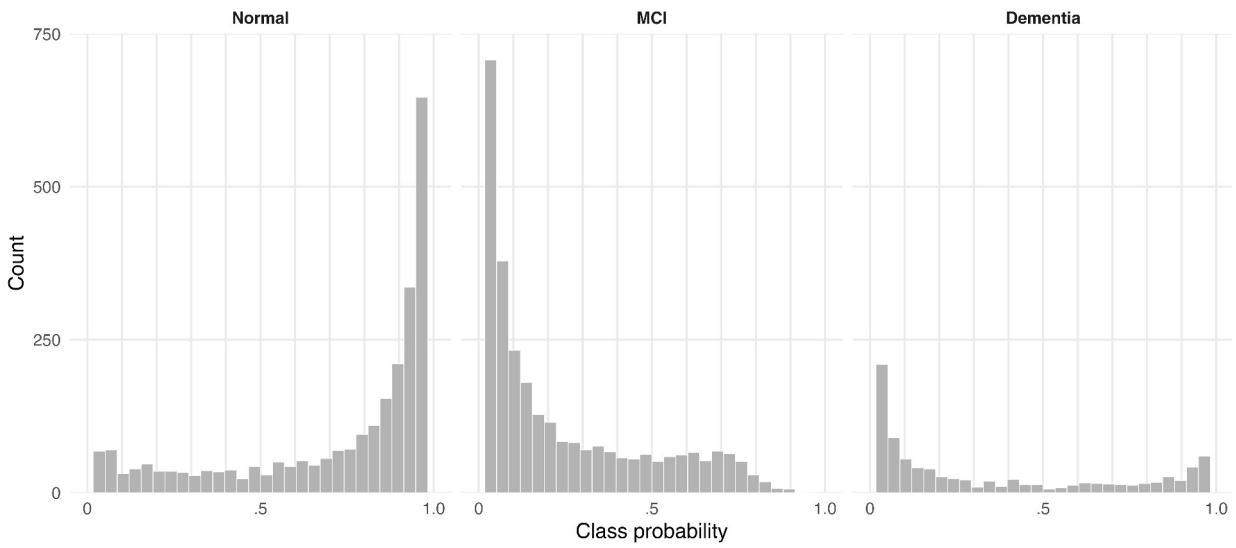


Figure 2: PMM class probabilities in HRS/HCAP

Thanks for reading.

Adaptation of the Manly-Jones algorithm to the HRS core survey data

2026-10-20

Purpose and overview

The Manly et al. (2022) algorithm classifies participants as having normal cognition, mild cognitive impairment (MCI), or dementia from 5 cognitive performance domain scores estimated from an hour-long neuropsychological battery, together with an informant interview on cognitive and functional change (Langa et al., 2020). None of that data is available in the core interview. The HRS core interview (Crimmins et al., 2011; Ofstedal et al., 2005), has a brief battery of scored cognitive items (8 used here), 1 self-rated memory item, and an informant questionnaire that is only administered when a proxy answers for the respondent and no cognitive performance data is available. These cognitive measures are too few to score separate domains defined in HCAP, and the lack of informant reports on the daily function of a respondent who answers for themselves generates an important additional difference in available information.

To address these differences, we constructed an adaptation we call the “core algorithm”, that follows the logic of the Manly-Jones algorithm with core measures and is compared with the Manly-Jones classifications of the same people at HCAP Wave 1 (2016). The core algorithm has 4 steps (Figure A1):

(1) a single cognitive factor score is estimated from the core items;

(2) the factor score is normed against the HRS-HCAP robust normative sample and expressed as a demographically adjusted T score;

(3) 2 T-score cut points divide the T score into 3 bands that stand in for the number of impaired domains; and

(4) decision rules combine the T-score band with self-rated memory change, or use the informant questionnaire when no cognitive tests were completed.

Item parameters and norms were estimated once using core data in the HRS-HCAP Wave 1 sample. These estimates and the cut points were then held fixed and used for the classification of participants in the 2016, 2018, 2020, and 2022 core interviews. A change in a participant’s classification across waves will reflect changes in that participant’s responses and/or age, but the scoring model does not change. The same item thresholds, norms, and cut points convert a 2022 response pattern exactly as they would convert the same pattern observed in 2016.

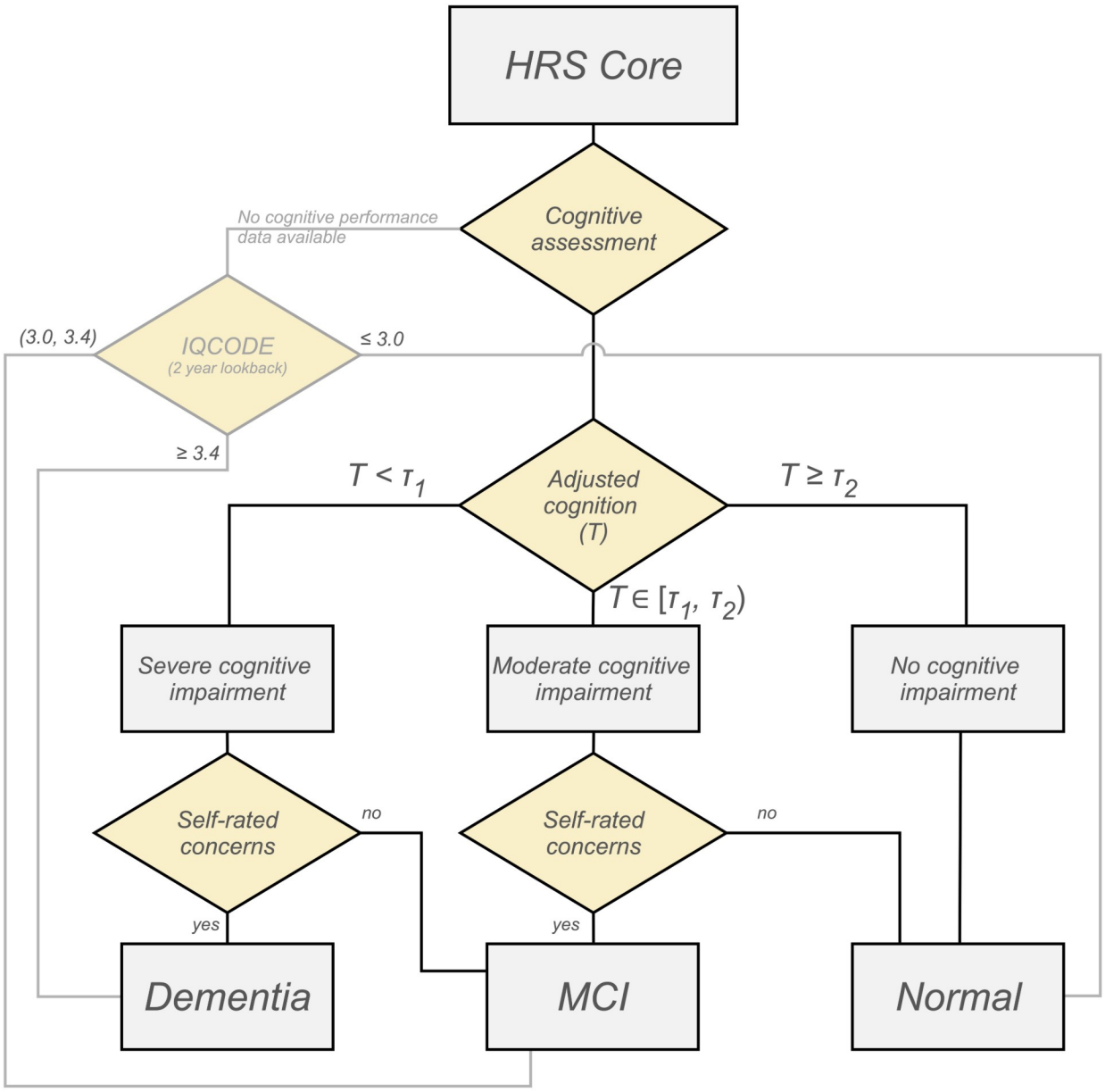


Figure A1. Core algorithm decision structure. T: demographically adjusted T score (Step 2); τ_1 and τ_2: severe and moderate T-score cut points (Step 3). IQCODE: Informant Questionnaire on Cognitive Decline in the Elderly. Self-rated concerns is a rating of memory worse than 2 years ago, and the "yes" arm includes don't know, refused, and not ascertained.

Data

The analytic sample is the 3,496 HRS-HCAP Wave 1 (2016) participants, followed in the HRS core interviews of 2016, 2018, 2020, and 2022. The core algorithm uses 4 kinds of core data:

(1) the 8 cognitive items in Table A1;

(2) self-rated memory compared with 2 years ago (HRS item D102: better, about the same, or worse);

(3) the 16-item informant questionnaire (IQCODE; Jorm, 1994), scored as the mean item rating from 1 (much improved) to 5 (much worse) and collected in proxy interviews; and

(4) age, sex, race and ethnicity (non-Hispanic Black, Hispanic), and years of schooling.

The reference classification for the core algorithm is the Manly-Jones classification algorithm as obtained in the HRS-HCAP Wave 1 sample. The Langa-Weir classification from the 2016 core interview (Crimmins et al., 2011) is used as a comparator used in the manuscript's sensitivity analysis, and we report its agreement with the Manly-Jones algorithm alongside that of the core algorithm.

Approach: Step 1. A single cognitive score from the core battery

The core data can be used to generate a unidimensional composite of cognitive performance. We begin this with a single-factor confirmatory factor analysis (CFA) model. This model uses ordinal and binary items entered through probit thresholds, and the 3 continuous scores (animal naming, delayed word recall, and number series) were rescaled to the 0-1 range (continuous), all modeled as caused by a single general latent variable. The model was fit to HRS 2016 respondents aged 65 and older (N = 9,218) with the complex sample design and the HRS-HCAP Wave 1 weights. The use of HRS-HCAP Wave 1 weights means that those respondents not in the HRS-HCAP sample are included in the analysis with a weight of 0, so the estimates describe the population that HRS-HCAP represents (3,283 HRS-HCAP participants with at least 1 HRS core item response). We first evaluate model fit in the pairwise complete sample with the weighted least squares mean and variance adjusted (WLSMV) estimator fit using Mplus software (version 9, Muthén & Muthén, Los Angeles, CA). Then, item parameters were estimated by maximum likelihood with robust standard errors (MLR) and a probit link, which uses every available item response regardless of item missingness. The WLSMV approach is consistent with the missing completely at random assumption, whereas the MLR approach is consistent with the less restrictive missing at random assumption. In subsequent factor score estimation runs, all parameters were fixed at the HCAP Wave 1 maximum likelihood estimates. Expected a posteriori (EAP) factor scores (Bock & Mislevy, 1982) were estimated from the fixed model for every core person-wave from 2016 to 2022 with at least 1 item response (63,992 person-waves, all ages). As with our analysis of the HCAP battery, immediate word recall was

not included in the model because it adds little information beyond delayed recall in the HCAP Wave 1 sample (Jones et al., 2024).

Results: Step 1. A single cognitive score from the core battery

The single-factor model fit well (RMSEA = 0.035, 90% CI 0.031 to 0.039; CFI = 0.966; TLI = 0.952; SRMR = 0.047)[1] by the conventional guides. Standardized loadings in the MLR model ranged from 0.48 to 0.71 (Table A1), and that range indicates that no single item dominates the factor. Table A1 provides loadings from the MLR and WLSMV models. Loadings from the two estimators were similar for most items. The lower WLSMV loading for number series, the item with the most missing data (12% of HRS-HCAP participants with any core item response, against at most 1% for any other item), likely reflects how each estimator handles missing data. The higher WLSMV loading for object naming cannot be explained by missing data, which was nearly complete for that item. Instead, the difference may be related to the item's skewed response distribution (93% of HRS-HCAP participants named both objects correctly, weighted).

Table A1. HRS core cognitive items and standardized factor loadings

Item	Scoring	Loading (MLR, used for scoring)	Loading (WLSMV)
Orientation to date	Month, day, year, day of week; 0-4 correct (ordinal)	0.48	0.52
Animal naming	Animals named minus errors; rescaled to 0-1 (continuous)	0.60	0.63
Object naming (scissors, cactus)	0-2 correct (ordinal)	0.68	0.84
Naming the President and Vice President	0-2 correct (ordinal)	0.60	0.64
Delayed word recall	0-10 words; rescaled to 0-1 (continuous)	0.63	0.64
Number series	Score rescaled to 0-1 (continuous)	0.71	0.62
Serial 7 subtraction	0-5 correct (ordinal)	0.66	0.60
Backward counting from 20	Correct on first or second try (binary)	0.52	0.56

Single-factor model fit to HRS 2016 respondents aged 65 and older (N = 9,218) with HRS-HCAP weights (weight 0 outside HRS-HCAP). Loadings are standardized (STDYX). Continuous items were rescaled to 0-1 using the 2016 range. WLSMV fit: chi-square = 248.5 (df = 20), RMSEA = 0.035, CFI = 0.966, TLI = 0.952, SRMR = 0.047.

[1] RMSEA, root mean square error of approximation, estimates model misfit per degree of freedom in the population and does not compare the model with a baseline model. Lower values indicate better fit; values below 0.05 indicate close fit (Hu & Bentler, 1999; Steiger, 1990). CFI, comparative fit index, and TLI, Tucker-Lewis index, compare the fitted model with a baseline model in which the items are uncorrelated. Higher values indicate better fit; values of 0.95 or above indicate satisfactory fit (Bentler, 1990; Hu & Bentler, 1999; Tucker & Lewis, 1973). The TLI adds a penalty for model complexity. SRMR, standardized root mean square residual, is the average difference between the observed and model-implied correlations. Lower values indicate better fit; values of 0.08 or below indicate satisfactory fit (Asparouhov & Muthén, 2018; Hu & Bentler, 1999).

Approach: Step 2. Demographic norms

The Manly-Jones algorithm defines impairment as performance more than 1.5 standard deviations below the level expected for a person's age, sex, race, ethnicity, and education in a robust normative sample (Manly et al., 2022). Although we use different thresholds to define impairment (discussed below), we norm the single general cognitive domain score the same way we normed domain scores in Manly et al. (2022).

The HCAP 2016 robust normative sample (n = 1,787) is used to define "expected" cognitive performance levels. First, factor scores in the normative sample were converted to normal scores with the Blom (1958) rank transformation. Because this transformation is sample dependent, we derive a linking equation between Blom transformed scores and estimated factor scores using linear regression with a restricted cubic spline for the factor score (knots at the 5th, 35th, 65th, and 95th percentiles; Harrell, 2015). The resulting regression model can be used to convert any factor score, including scores from people outside the normative sample and from later waves, to the normalized metric.

Second, the normalized score was regressed, with survey weights, on age (restricted cubic spline with knots at 70, 78, 86, 94 years, centered at 70), sex, non-Hispanic Black race, Hispanic ethnicity, and years of schooling (centered at 12), together with their 2-way interactions (24 terms). This demographic equation is used to estimate the expected score according to demographic characteristics.

Third, the sociodemographically adjusted cognitive performance score (T score) was computed as 50 + 10 × (observed − expected) / SEE, where SEE, the standard error of the estimate from the demographic regression in the normative sample, is the standard deviation of the normalized score multiplied by $\sqrt{(1 - R^2)}$. A T score of 40 therefore means performance 1 SEE below the level expected for a person of the same age, sex, race, ethnicity, and education. The same SEE is used regardless of demographic group, so a given T score marks the same distance below expectation for every participant.

Results: Step 2. Demographic norms

The demographic equation explained 37% of the variance in the normalized score in the normative sample. The standard deviation of the normalized score was 0.973, and therefore the

SEE was 0.775 normalized-score units. In HRS-HCAP Wave 1, the unweighted mean (SD) T score was 50 (10) in the normative sample[2] and 43 (12) among participants excluded from it.

Approach: Step 3. Cut points

In the Manly-Jones algorithm, dementia requires at least 2 impaired domains and an informant report of functional impairment, and MCI requires at least 1 impaired domain with informant-reported concern or self-reported memory worsening (Manly et al., 2022).

No point on a single T score corresponds by construction to 2 or more domains more than 1.5 standard deviations below expectation. With 5 domains, there are 10 different ways to satisfy exactly 2 domains impaired, and each would be associated with a different cut score on the single domain. No rule fixes in advance where on the T scale the cut points belong.

We used 2 cut points on the T score (T representing the single domain cognitive performance measure, adjusted for sociodemographics and rescaled to a T distribution (M 50 SD 10) in the unweighted normative reference group): a severe cut point (τ_1) at T < 37 and a moderate cut point (τ_2) at T < 48. The 2 cut points stand in for the count of impaired domains: T below τ_1 corresponds to impairment in 2 or more domains, and T from τ_1 to below τ_2 corresponds to impairment in 1 domain.

The severe cut point lies above the T score below which the weighted share of HRS-HCAP Wave 1 participants equals the share with 2 or more impaired domains by the Manly-Jones algorithm (0%; T ≈ -6; n = 2,786). The moderate cut point lies above the T score that matches the share with 1 or more impaired domains (22%; T ≈ 42). A higher moderate cut point is needed because the core rules classify participants below τ_2 who do not report memory worsening as normal (Step 4), which lowers the MCI share.

Results: Step 3. Cut points

Figure A2 shows the weighted distribution of T scores among HRS-HCAP Wave 1 participants with core cognitive tests, with the 2 cut points and the weighted share of participants in each T-score band.

[2] Manly et al. (2022) used the unweighted normative sample in this scale calibration step. An alternative would have been to use the sampling weights in the normative reference subpopulation group to set the mean (50) and standard deviation (10) on the rescaled cognitive performance measure. We use the convention established in Manly et al. (2022).

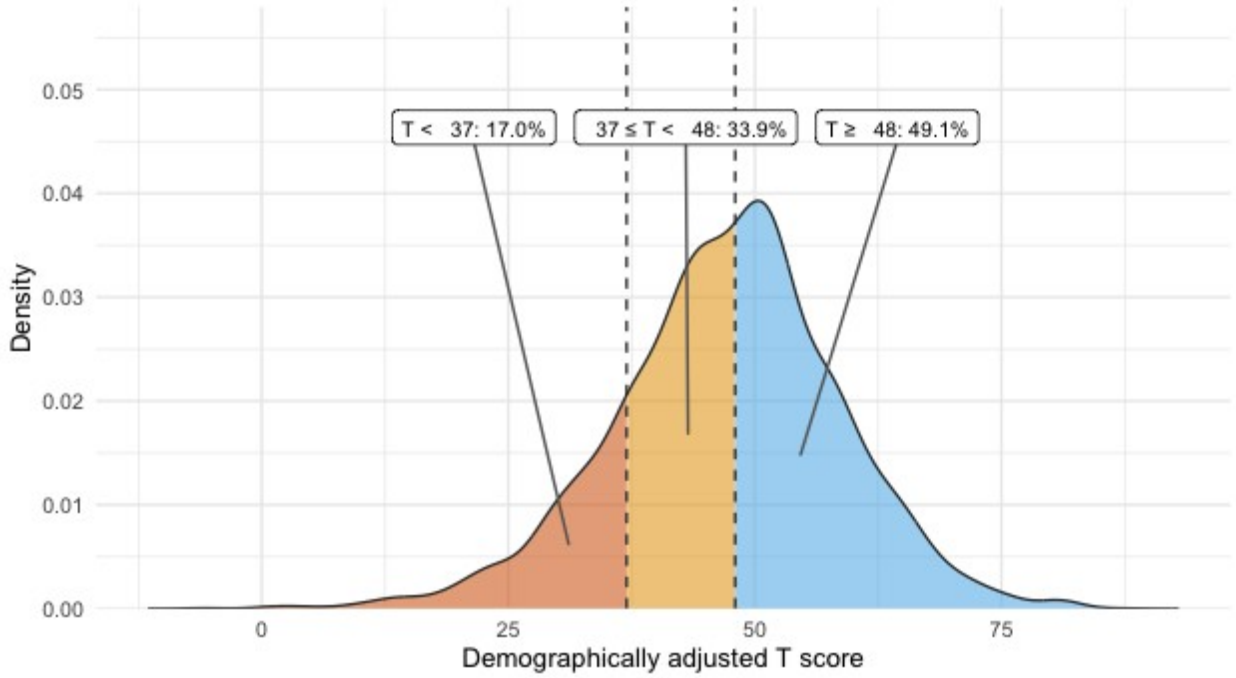


Figure A2. Weighted distribution of demographically adjusted T scores among HRS-HCAP Wave 1 participants with core cognitive tests (n = 3,283), with the cut points (dashed lines). Band labels give the weighted share of these participants in each T-score band.

Step 4. Classification rules

The core interview has no informant report for respondents who participated in the cognitive assessment. We replaced each element of the Manly-Jones rules with its closest core counterpart (Figure A1; Table A2).

The core interview asks every self-respondent about limitations in activities of daily living (ADL) and instrumental activities of daily living (IADL), and these self-reports could stand in for the informant report of functional impairment that separates dementia from MCI in the Manly-Jones algorithm. We did not use them, for 2 reasons. First, few participants report any limitation, even among those the Manly-Jones algorithm classifies as having dementia. Among HRS-HCAP Wave 1 participants with a core T score and dementia by the Manly-Jones algorithm, 52.3% (weighted) reported no ADL or IADL limitation, so no threshold on the limitation count can identify them. Second, the core items do not say why a limitation is present. Limitations from physical illness are common at these ages, and a rule that counts them would move participants with low T scores into the dementia class on the strength of a physical disability. The Endnote gives the full evidence and compares alternative rules at the core algorithm's cut points.

The second criterion at both levels is therefore self-rated memory worsening, an item the Manly-Jones algorithm already includes, used in place of the informant report. Participants without

core cognitive tests (mostly proxy interviews) were classified from the IQCODE with the Manly-Jones informant thresholds (3.4 or higher for dementia; above 3.0 for MCI). Missing self-rated memory was singly imputed by chained equations (van Buuren & Groothuis-Oudshoorn, 2011); no Wave 1 participant with a T score was missing self-rated memory, and “don’t know,” refused, and not ascertained responses (4 participants) were counted as “worse”, which assigns these participants to the more impaired class.

Table A2. Core algorithm decision rules and participants classified by each rule, HRS-HCAP Wave 1

Route	Cognitive criterion	Second criterion	Classification	n
Cognitive tests	T < 37	Memory worse than 2 years ago*	Dementia	209
Cognitive tests	T < 37	Memory better or the same	MCI	390
Cognitive tests	37 ≤ T < 48	Memory worse than 2 years ago*	MCI	320
Cognitive tests	37 ≤ T < 48	Memory better or the same	Normal	816
Cognitive tests	T ≥ 48	Any	Normal	1,548
Proxy interview	No T score	IQCODE ≥ 3.4	Dementia	107
Proxy interview	No T score	3.0 < IQCODE < 3.4	MCI	37
Proxy interview	No T score	IQCODE ≤ 3.0	Normal	69

n: unweighted HRS-HCAP Wave 1 participants (N = 3,496; Normal 2,433, MCI 747, Dementia 316). T: demographically adjusted T score (mean 50, SD 10 in the HRS-HCAP robust normative sample). *Self-rated memory compared with 2 years ago (HRS item D102); don't know, refused, and not ascertained count as worse; missing values were singly imputed. IQCODE: 16-item Informant Questionnaire on Cognitive Decline in the Elderly, mean item score (1-5), asked in proxy interviews.

With these rules, the weighted shares of the core algorithm in HRS-HCAP Wave 1 were 71.0% normal, 20.7% MCI, and 8.3% dementia, compared with 69.4%, 21.4%, and 9.2% for the Manly-Jones algorithm estimated with EAP factor scores (Table A3).

Table A3. Weighted class shares of the core algorithm and the Manly-Jones algorithm, HRS-HCAP Wave 1

Classification	Normal, %	MCI, %	Dementia, %
Core algorithm	71.0	20.7	8.3
Manly-Jones algorithm	69.4	21.4	9.2

HRS-HCAP Wave 1, N = 3,496. Percentages are weighted (HCAP Wave 1 (2016) sampling weight). Core algorithm cut points: T < 37 (severe) and T < 48 (moderate).

Agreement with the Manly-Jones algorithm in HRS-HCAP Wave 1

We next examine the agreement of the core algorithm with the Manly-Jones classification and compare it with the agreement of the Langa-Weir classification, a widely used classification of cognitive status from HRS core data (Crimmins et al., 2011). For self-respondents, Langa-Weir

sums immediate and delayed word recall (0 to 20), serial 7 subtraction (0 to 5), and backward counting (0 to 2) into a 27-point score, and classifies scores of 0 to 6 as dementia, 7 to 11 as cognitive impairment without dementia (CIND), and 12 to 27 as normal. For proxy interviews, it combines the proxy's report of the respondent's memory, limitations in instrumental activities of daily living, and the interviewer's assessment of cognitive impairment into an 11-point score, with 6 or more classified as dementia and 3 to 5 as CIND (cognitive impairment, not demented; Crimmins et al., 2011, pp. i164, i166). The Langa-Weir cut points were chosen so that the HRS distribution of cognitive states matches the distribution estimated in the Aging, Demographics, and Memory Study (ADAMS). The Langa-Weir score is not adjusted for demographic characteristics. We treat CIND as the counterpart of MCI.

Agreement of the core algorithm with the Manly-Jones algorithm for 3-class Normal/MCI/Dementia classification was about the same as the agreement of Langa-Weir; for dementia versus not, it was lower than that of Langa-Weir (Table A4). The quadratic weighted kappa (Cohen, 1968) for the 3 classes was 0.50 for the core algorithm and 0.51 for Langa-Weir; for dementia versus not, kappa was 0.35 and 0.41. Of the weighted population the Manly-Jones algorithm classified as having dementia, the core algorithm classified 39% as having dementia and Langa-Weir classified 42%. Of those the core algorithm classified as having dementia, 43% had dementia by the Manly-Jones algorithm (Langa-Weir: 50%). Agreement between the core algorithm and Langa-Weir was 0.56 (3 classes, weighted kappa) and 0.44 (dementia, kappa). Table A5 gives the weighted cross-classifications. Each kappa compares 2 classifications that both contain error, so these values describe agreement and not accuracy.

Table A4. Agreement of the core algorithm and Langa-Weir with the Manly-Jones algorithm, HRS-HCAP Wave 1

Comparison	Kappa, 3-class (quadratic)	Kappa, dementia vs. not	Manly-Jones dementia classified as dementia, %	Comparator dementia that is Manly-Jones dementia, %
Core algorithm vs. Manly-Jones	0.50	0.35	39	43
Langa-Weir vs. Manly-Jones	0.51	0.41	42	50
Core algorithm vs. Langa-Weir	0.56	0.44	n/a	n/a

N = 3,496 HRS-HCAP Wave 1 participants with all 3 classifications; statistics weighted with HCAP Wave 1 (2016) sampling weight. Core algorithm and Langa-Weir use 2016 HRS core data; the Manly-Jones algorithm uses HRS-HCAP data. Kappa, 3-class: Normal, MCI, Dementia with quadratic weights (Cohen, 1968). The last 2 columns treat the Manly-Jones classification as the reference; n/a, not applicable.

Table A4. Agreement of the core algorithm and Langa-Weir with the Manly-Jones algorithm, HRS-HCAP Wave 1

Comparison	Kappa, 3-class (quadratic)	Kappa, dementia vs. not	Manly-Jones dementia classified as dementia, %	Comparator dementia that is Manly-Jones dementia, %

Table A5. Weighted cross-classification of the core algorithm and Langa-Weir with the Manly-Jones algorithm, HRS-HCAP Wave 1

Comparator	Manly-Jones class	Comparator class: Normal	Comparator class: MCI	Comparator class: Dementia	Total
Core algorithm	Normal	29,237 (84%)	4,750 (14%)	802 (2%)	34,788
Core algorithm	MCI	5,462 (51%)	3,712 (35%)	1,551 (14%)	10,725
Core algorithm	Dementia	890 (19%)	1,933 (42%)	1,797 (39%)	4,620
Langa-Weir	Normal	30,246 (87%)	4,091 (12%)	450 (1%)	34,788
Langa-Weir	MCI	6,070 (57%)	3,120 (29%)	1,535 (14%)	10,725
Langa-Weir	Dementia	1,220 (26%)	1,439 (31%)	1,961 (42%)	4,620

Rows: Manly-Jones class (HRS-HCAP). Columns: comparator class (core algorithm or Langa-Weir) from 2016 HRS core data. Cell entries are survey-weighted population counts in thousands. Entries for fewer than 1,000 persons show two significant digits. Row percentages in parentheses. N = 3,496 participants with both classifications (HCAP Wave 1 (2016) sampling weight). Kappas are in Table A4.

Application to the 2018, 2020, and 2022 core interviews

The fixed model, norms, and cut points were applied to every core interview of HRS-HCAP Wave 1 participants from 2016 to 2022 (Table A6). The number of classified person-waves fell from 3,496 in 2016 to 2,049 in 2022 as participants died or left the study, and the share classified from the IQCODE alone was 6.1% in 2016 and 6.9% in 2022.

Table A6. Core algorithm classifications of HRS-HCAP Wave 1 participants interviewed in the HRS core, 2016-2022

Core wave	Person-waves	Cognitive tests	IQCODE only	Not classified	Normal	MCI	Dementia
2016	3,496	3,283	213	0	2,433 (69.6%)	747 (21.4%)	316 (9.0%)
2018	2,989	2,836	152	1	2,163 (72.4%)	603 (20.2%)	222 (7.4%)
2020	2,558	2,380	178	0	1,920 (75.1%)	450 (17.6%)	188 (7.3%)
2022	2,050	1,907	142	1	1,474 (71.9%)	392 (19.1%)	183 (8.9%)

Unweighted counts of person-waves; percentages among classified person-waves. Not classified: no cognitive tests and no IQCODE. Number series was not administered in the 2018 core interview, so 2018 T scores rest on 7 items.

One wave departs from the others. Number series was not administered in the 2018 core interview (no 2018 respondent has a score), so 2018 EAP scores are based on 7 items. With less information, an EAP score is pulled further toward the population mean (Bock & Mislevy, 1982), which raises the scores of low performers and may classify fewer participants as

impaired in 2018 than the same performance would produce in other waves. We did not re-norm the 7-item score because the 7 items are in common with the 2016 core items and factor scores are estimated with fixed parameter estimates, therefore the factor score estimates are on the same scale (fixed parameter calibration).

Figure A3 and Table A7 compare EAP scores of core respondents aged 65 or older in 2018 (when number series was not administered) with those in 2016 and 2020 (combined, when number series was administered). The mean posterior standard deviation of the EAP score (the uncertainty in one participant's score given that participant's item responses, averaged over participants) was 0.56 in 2018 and 0.49 in 2016 and 2020, and the standard deviation of the EAP scores (the spread of the score estimates across participants) was 0.85 and 0.89. Under the model, the variance of the EAP scores and the mean posterior variance sum to the factor variance of 1 (here 1.05 in 2018 and 1.03 in 2016 and 2020), so the larger posterior variance in 2018 appears as a narrower distribution of EAP scores. The lower quartile was -0.60 in 2018 and -0.65 in 2016 and 2020. The effect on the spread of scores was small; the standard deviation of the 2018 EAP scores was about 4% smaller than in 2016 and 2020 (Table A7). A small shift for the population can still change the classification of individuals whose scores lie near a cut point. Because shrinkage moves low scores toward the mean, a participant whose T score would fall just below τ_1 with number series may fall just above it without it, and be classified with MCI rather than dementia.

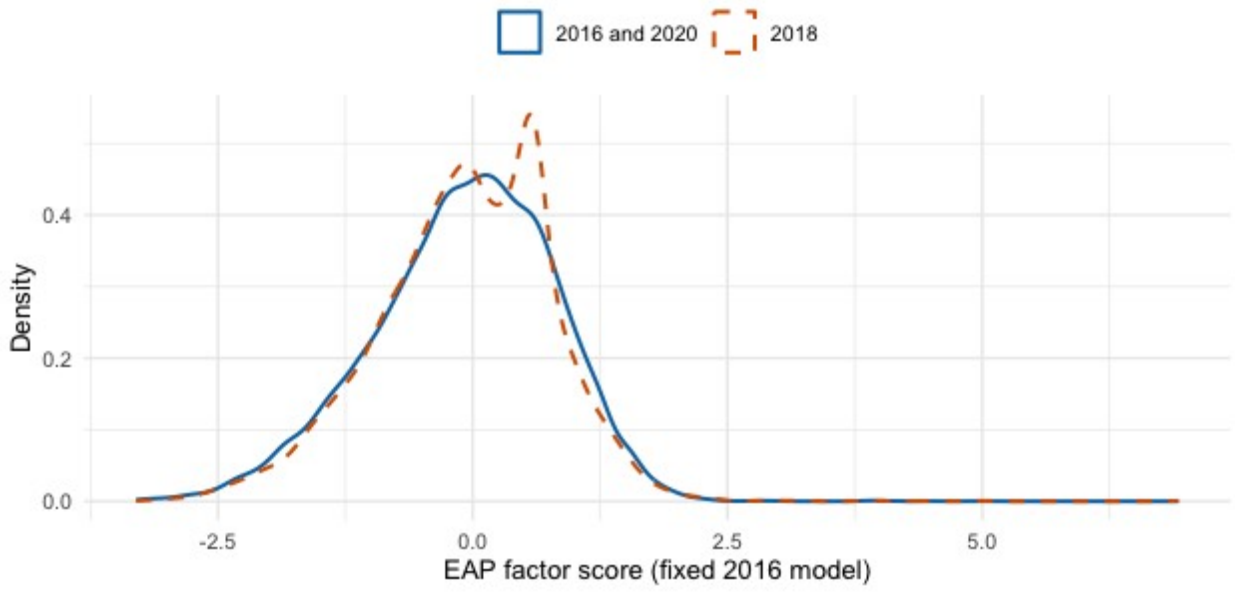


Figure A3. Distribution of EAP factor scores among core respondents aged 65 or older with a nonzero wave weight, in waves with number series (2016 and 2020) and without it (2018). Scores come from the fixed 2016 model.

Table A7. EAP factor scores in core waves with and without number series

Core waves	Person-waves	Minimum	Q1	Median	Q3	Maximum	SD of EAP	Mean posterior SD	Var(EAP) + mean posterior variance
2016 and 2020	17,518	-3.30	-0.65	-0.02	0.55	5.01	0.89	0.49	1.03
2018	8,404	-3.26	-0.60	-0.03	0.59	6.93	0.85	0.56	1.05

Core respondents aged 65 or older with a nonzero wave weight and at least 1 cognitive item; unweighted. 2016 and 2020: number series administered; 2018: not administered. 2022 is omitted because number series is missing for most face-to-face interviews. Under the model, the variance of the EAP scores plus the mean posterior variance equals the factor variance (1), so a larger posterior SD implies a smaller spread of EAP scores (shrinkage toward the mean).

Table A8 lists the number of core respondents aged 65 or older with a score on each test, by wave. Number series was present for 84% (weighted) of respondents in 2016, 86% in 2020, and 49% in 2022, and absent in 2018; the other tests were present for at least 78% of respondents in every wave.

Table A8. Core cognitive tests with a score, by wave: number (weighted %)

Test	2016	2018	2020	2022
Orientation to date	9,158 (94.9%)	7,578 (83.8%)	7,219 (78.3%)	6,751 (79.4%)
Animal naming	9,092 (94.2%)	7,404 (82.0%)	7,218 (78.3%)	6,716 (79.0%)
Object naming	9,158 (94.9%)	7,578 (83.8%)	7,219 (78.3%)	6,751 (79.4%)
President and Vice President	9,157 (94.8%)	7,578 (83.8%)	7,219 (78.3%)	6,751 (79.4%)
Delayed word recall	9,094 (94.1%)	7,493 (82.9%)	7,157 (77.5%)	6,641 (78.1%)
Number series	7,875 (84.2%)	0 (0.0%)	7,274 (85.8%)	3,897 (49.4%)
Serial 7 subtraction	9,158 (94.9%)	8,404 (95.7%)	8,360 (94.8%)	7,759 (95.4%)
Backward counting from 20	9,121 (94.5%)	7,535 (83.4%)	7,194 (77.9%)	6,725 (79.1%)
Immediate word recall (not in model)	9,095 (94.2%)	7,514 (83.1%)	7,185 (77.8%)	6,691 (78.7%)
Any cognitive item	9,158 (94.9%)	8,404 (95.7%)	8,383 (95.1%)	7,759 (95.4%)

Core respondents aged 65 or older with a nonzero wave weight (2016: n = 9,684; 2018: n = 8,822; 2020: n = 8,869; 2022: n = 8,161). Counts are unweighted; percentages are weighted with the wave respondent weight. Proxy interviews, which have no cognitive tests, are included in the denominator.

Limitations

Limitations of our approach are worth mentioning.

First, the cut points may have been chosen with reference to the HRS-HCAP Wave 1 sample, in which agreement is also evaluated, so the agreement in Table A4 may be optimistic. The

comparison of core-based and HRS-HCAP-based classifications among Wave 2 re-interviewees (eTables 7 and 8) is the out-of-sample check available in these data.

Second, self-rated memory replaces informant report, and most participants with low T scores did not report memory worsening; the core algorithm likely under-classifies dementia relative to the Manly-Jones algorithm, which is consistent with the lower core-based dementia incidence among re-interviewees in eTable 8.

Third, the 2018 scores rest on 7 items (above). The pattern of missingness for number series is difficult to discern from the data and the HRS concordance web tool.

Fourth, the norms do not account for interview mode, which varies across respondents and waves; a mode difference in test scores would shift T scores in the waves where the mode mix differs from 2016. Other potentially important considerations, such as language of administration, are not incorporated into the scoring and standardizing steps.

Fifth, the IQCODE in the core interview asks about change over 2 years, whereas the HRS-HCAP informant interview asks about change over 10 years, so the Manly-Jones IQCODE thresholds may not carry the same meaning in core proxy interviews.

Finally, both the core algorithm and the Manly-Jones algorithm rely on applying strict cut points to continuous cognitive, functional, and informant report measures. This practice of attempting to elicit qualitative clinical states from quantitative scales introduces well-documented classification instability, particularly for individuals whose scores fall near diagnostic thresholds (Streiner, 2002; Altman & Royston, 2006). Because continuous cognitive instruments inherently possess standard measurement error, small and clinically negligible fluctuations in test performance can shift an individual across an arbitrary boundary. As a result, observations that are quantitatively almost indistinguishable are operationalized as qualitatively distinct diagnostic categories, substantially inflating misclassification rates and generating spurious diagnostic changes across repeated assessments.

Endnote. Why the core algorithm does not use functional limitations

Our original plan followed the Manly-Jones algorithm more closely. In the Manly-Jones algorithm, an informant report of functional impairment separates dementia from MCI (Manly et al., 2022), and the core interview asks every self-respondent about limitations in activities of daily living (ADL) and instrumental activities of daily living (IADL). We planned to use these self-reports in place of the informant report and to set their thresholds by prevalence matching within

each level of cognitive impairment. We set the plan aside for 3 reasons, which we document here with the current data. The first and third reasons are summarized in Step 4. Limitations were counted on the 9 ADL and IADL items asked in every core wave from 2016 to 2022 (Table A9 note), so that the count would mean the same thing in every wave.

First, few participants report any limitation, even among those the Manly-Jones algorithm classifies as having dementia. Among HRS-HCAP Wave 1 participants with a core T score, the weighted share reporting no ADL or IADL limitation was 52.3% for those with dementia by the Manly-Jones algorithm, 68.7% for MCI, and 84.0% for normal cognition; below the severe T cut point, 64.0% reported none (Table A9). By definition, every participant the Manly-Jones algorithm classifies as having dementia has an informant report of functional impairment, yet 52.3% of them report no limitation in the core interview. The lowest possible threshold, 1 or more limitations, misses at least that share, and no threshold on the limitation count can do better.

Second, adding function raised agreement by a small amount, and the gain held in nearly every random half of the sample. We applied 2 alternative rules, all at the core algorithm's cut points ($T < 37$ and $T < 48$), so that the rules differ only in the second criterion: function at the severe level and function or memory at the moderate level, the closest analog of the Manly-Jones rules; and function or memory at both levels (Table A10). In the full sample, the 3-class kappa was 0.50 with the memory rule, 0.53 with the function rule, and 0.53 with function or memory; the dementia kappa was 0.35, 0.40, and 0.41. Because the cut points are held fixed, the rules also produce different class shares (Table A10). To describe how much these differences vary across samples, we computed agreement in each of 200 random halves of the sample (Table A11). Relative to the memory rule, the mean difference in 3-class kappa across halves was +0.03 (0.00 to +0.05) for the function rule and +0.02 (+0.01 to +0.04) for function or memory (2.5th to 97.5th percentile across halves), and the function rule had the higher kappa in 99% of halves. For dementia, the mean differences were +0.05 (-0.01 to +0.11) and +0.05 (+0.02 to +0.10). The gain is therefore small but consistent across halves, and our decision rests on the third reason.

Third, the core items do not say why a limitation is present. A participant who cannot bathe without help because of hip arthritis and a participant who cannot bathe without help because they no longer remember the steps give the same answer. Limitations from physical illness are common at the ages of HRS-HCAP participants, and a rule that counts any limitation would move some participants with low T scores into the dementia class on the strength of a physical

disability. We judged that the gain in agreement was not large enough to justify letting limitations with probable physical causes enter the dementia classification. We kept the memory rule, which uses an item the Manly-Jones algorithm already includes. An earlier version of this analysis, with different cut points and an unweighted subsample, found the same floor: within each level of cognitive impairment, fewer participants reported any limitation than had functional impairment by the Manly-Jones algorithm, so no threshold on the limitation count could match the HRS-HCAP rates.

Table A9. Self-reported ADL and IADL limitations by Manly-Jones class and by core T-score band, HRS-HCAP Wave 1

Grouping	Group	n	None, %	1, %	2 or more, %	Missing, %
Manly-Jones class	Normal	2,256	84.0	8.8	7.2	0.0
Manly-Jones class	MCI	744	68.7	14.2	17.1	0.0
Manly-Jones class	Dementia	283	52.3	14.1	33.6	0.0
Core T-score band	T < 37	599	64.0	12.1	23.9	0.0
Core T-score band	37 to < 48	1,136	78.3	9.8	11.9	0.0
Core T-score band	T >= 48	1,548	83.7	10.0	6.2	0.0

Participants with a core T score (n = 3,283). Number of limitations in activities of daily living (ADL) and instrumental activities of daily living (IADL) reported in the 2016 core interview, counted on the 9 items asked in every wave from 2016 to 2022 (dressing, bathing, eating, and using the toilet; preparing meals, shopping for groceries, using the telephone, taking medications, and managing money). Missing: no response to any of these items. Percentages are weighted (HCAP Wave 1 (2016) sampling weight).

Table A10. Agreement with the Manly-Jones algorithm when functional limitations enter the decision rules, HRS-HCAP Wave 1

Rule for the second criterion	Severe cut	Moderate cut	MCI, %	Dementia, %	Kappa, 3-class	Kappa, dementia	Manly-Jones dementia classified as dementia, %
Memory worse (core algorithm)	37	48	20.7	8.3	0.50	0.35	39
Function at severe level; function or memory at moderate level	37	48	24.4	8.8	0.53	0.40	44
Function or memory at both levels	37	48	21.6	11.5	0.53	0.41	53

N = 3,496. All rules use the core algorithm's cut points (T < 37 and T < 48), so the rules differ in class shares as well as agreement. Function: 1 or more ADL or IADL limitations on the items in Table A9 (missing counted as none). Memory: memory worse than 2 years ago. Statistics are weighted (HCAP Wave 1 (2016) sampling weight); kappa, 3-class uses quadratic weights. Table A11 shows how these statistics vary across random halves of the sample.

Table A9. Self-reported ADL and IADL limitations by Manly-Jones class and by core T-score band, HRS-HCAP Wave 1

Grouping	Group	n	None, %	1, %	2 or more, %	Missing, %

Table A11. Agreement with the Manly-Jones algorithm in repeated random halves, HRS-HCAP Wave 1

Rule for the second criterion	Kappa, 3-class	Difference from memory rule	Halves higher, %	Kappa, dementia	Difference from memory rule	Halves higher, %
Memory worse (core algorithm)	0.50 (0.46, 0.54)	n/a	n/a	0.35 (0.29, 0.41)	n/a	n/a
Function at severe level; function or memory at moderate level	0.53 (0.49, 0.57)	+0.03 (0.00, +0.05)	99	0.40 (0.34, 0.46)	+0.05 (-0.01, +0.11)	94
Function or memory at both levels	0.53 (0.49, 0.57)	+0.02 (+0.01, +0.04)	99	0.41 (0.35, 0.46)	+0.05 (+0.02, +0.10)	100

200 random halves of the sample, drawn within Manly-Jones class. In each half, agreement was computed for each rule at the core algorithm's cut points (T < 37 and T < 48). Entries are the mean across halves (2.5th and 97.5th percentiles). Differences are paired within half. The halves overlap, so the percentiles describe variation across halves and are not confidence intervals. Statistics are weighted (HCAP Wave 1 (2016) sampling weight); kappa, 3-class uses quadratic weights.

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Appendix 2. PMM and logistic regression

2026-10-20

PMM and logistic regression can both classify people into Normal, MCI, or Dementia. The difference is what they model.

Logistic regression

Logistic regression starts with a known outcome, Y, and predicts it from observed predictors, X:

$$P(Y = \text{Dementia} \mid X)$$

For example, it could predict dementia from word recall, serial sevens, ADLs, and Jorm score. It estimates how each predictor changes the log-odds of each outcome category. In a multinomial logistic model, the usual form is:

$$\log \frac{P(Y = k \mid X)}{P(Y = \text{Normal} \mid X)} = \beta_{0k} + \beta_{1k} X_1 + \cdots + \beta_{pk} X_p$$

The main modeling decision is the regression equation: which predictors, interactions, nonlinear terms, and coefficients to include.

PMM in this project

The PMM takes a generative approach. It asks: *If a person belongs to the Normal, MCI, or Dementia class, what pattern of cognitive-test, functional, self-report, and informant responses would we expect?* PMM models:

$$P(X \mid C = k)$$

for each cognitive class $C = k$, along with the class prevalence $P(C = k)$. It then uses Bayes' rule to obtain the classification probability:

$$P(C = k \mid X) = \frac{P(X \mid C = k) P(C = k)}{\sum_j P(X \mid C = j) P(C = j)}$$

So, instead of directly predicting diagnosis from scores, it describes the characteristic response distribution within each diagnosis class and asks which class best accounts for a person's observed pattern.

For the final PMM, the cognition model uses the individual test scores, functional items, and self-report item directly. It estimates class probabilities from that whole response pattern. The separate Jorm PMM does the analogous job for people with informant data rather than cognitive testing.

Practical distinctions

- Logistic regression: “Given these scores, what is the probability of Dementia?”
- PMM: “Which diagnosis class most plausibly generated this pattern of scores and reports?”
- Logistic regression is discriminative: it models $P(Y \mid X)$ directly.
- PMM is generative: it models $P(X \mid C)$, then derives $P(C \mid X)$.
- Logistic regression commonly treats predictors as explanatory variables with regression coefficients.
- PMM treats the observed responses as indicators of an underlying class-specific response profile.
- Logistic regression needs explicit interactions or nonlinear terms when the meaning of one score depends on another.
- A PMM can represent distinct multivariate profiles through class-specific item distributions, although it still relies on assumptions about within-class dependence.

In this project, the important wrinkle is that the PMM is not purely exploratory latent class analysis. It is calibrated using the HRS/HCAP Algorithm classification as the known class in the HCAP sample, then scores the Core measures of the same HCAP participants with the calibrated parameters (including the class proportions) held fixed. That makes it a supervised, model-based classifier: conceptually closer to generative discriminant analysis than to an unsupervised profiling exercise.

A concise manuscript description would be:

> *The PMM is a supervised, generative classifier that estimates the probability of each HRS/HCAP Algorithm class from the joint pattern of observed cognitive, functional, self-report, or informant responses.*

Appendix 3: Comparisons with other study classifications

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